This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the supply of Varenicline 0.5mg and 1mg tablets for the treatment of Smoking Cessation

Patient Group Direction, version 006, issued 30 September 2026. Supersedes Version 005, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE / NICE CKS Guidelines for Smoking Cessation

Overview

- Smoking is the leading preventable cause of death
- Nicotine dependence is a chronic relapsing condition
- Pharmacotherapy (varenicline, NRT, bupropion) doubles quit rates compared to placebo

Assessment

- Fagerström score to assess nicotine dependence
- Assess readiness to quit and motivation
- Set a quit date (typically 1-2 weeks from assessment)
- Review previous quit attempts and reasons for relapse
- Screen for contraindications and red flags

Management

- Varenicline is first-line pharmacotherapy for smoking cessation

- Start 1-2 weeks before quit date to achieve steady state
- Combine with structured behavioural support for optimal outcomes
- Follow-up at 2 weeks, 4 weeks, then monthly during treatment

Red Flags

- **Psychiatric history:** Depression, anxiety disorder, psychosis, bipolar disorder - varenicline can exacerbate mood symptoms
- **Suicidal ideation:** Assess risk; varenicline is associated with neuropsychiatric effects
- **Cardiovascular disease:** Recent MI (within 4 weeks), unstable angina, severe arrhythmias - assess benefit vs risk

Patient Group Direction

for the supply/administration of

Varenicline (as tartrate) 0.5mg and 1mg film-coated tablets. Any UK-licensed generic varenicline product; Champix is no longer marketed in the UK.

for the treatment of

Smoking Cessation

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing

	organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Smoking cessation in adults aged 18 years and over who have set a quit date
Inclusion criteria	Age 18 years and over Motivated to quit smoking Set a quit date 8 to 14 days after the planned start of treatment (within the next 2 weeks where treatment starts on the day of supply) Capacity and willingness to provide informed consent
Exclusion criteria	Known hypersensitivity to varenicline or excipients Pregnancy or breastfeeding Severe renal impairment (eGFR below 30 mL/min/1.73m2) or end-stage renal disease. Refer to GP. Age under 18 years
Cautions	Psychiatric history: depression, anxiety disorder, psychosis, or bipolar disorder (varenicline can exacerbate mood symptoms; monitor closely) History of suicidal ideation or self-harm (monitor mental health throughout treatment) Cardiovascular disease: recent MI (within 4 weeks), unstable angina, or severe cardiac arrhythmias (assess benefit vs risk) Seizure disorders (varenicline may lower seizure threshold; rare but serious) Renal impairment, eGFR 30 to 50 mL/min/1.73m2: no dose adjustment; if adverse effects are not tolerated reduce to 1 mg once daily. eGFR below 30 excludes.

	Driving or operating heavy machinery (varenicline may cause dizziness, somnolence, or impaired concentration)
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Varenicline (as tartrate) 0.5mg and 1mg film-coated tablets. Any UK-licensed generic varenicline product; Champix is no longer marketed in the UK.
Legal category	POM
Storage	Below 30°C. Protect from light. Keep in original packaging.
Route/method of administration	Oral, swallowed whole with water. May be taken with or without food.
Dose and frequency	Days 1-3: 0.5mg once daily Days 4-7: 0.5mg twice daily (morning and evening) Day 8 to end of treatment: 1mg twice daily (morning and evening) Quit date should be on day 8-14 of treatment (when 1mg twice daily is established) Standard course: 12 weeks. Consider additional 12 weeks (total 24 weeks) for sustained abstinence to reduce relapse risk
Quantity to be administered and/or supplied	Starter pack covering the first 2 weeks (11 x 0.5mg and 14 x 1mg tablets), then up to 56 tablets of 1mg per supply (4-week supply at 1mg twice daily). Supply no more than covers the period to the next review.
Maximum or minimum treatment period	12 weeks; may extend to 24 weeks total. Review at 2 weeks, 4 weeks, then monthly.
Adverse effects	Very common (≥1/10): Nausea (dose-related, usually transient and improves with time; may take with food if needed); insomnia; abnormal dreams Common (≥1/100, <1/10): Headache; somnolence; dizziness; dry mouth; constipation; flatulence; vomiting; dyspepsia; fatigue Uncommon (≥1/1000, <1/100): Mood changes; sleep disturbance; appetite changes Rare (serious): Depression; suicidal ideation (monitor closely throughout treatment); cardiovascular events (MI, stroke); seizures;

	angioedema

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply, dose, form, route and quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken • that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years

Patient information

Written information to be given to patient or carer	Supply the Patient Information Leaflet provided with the varenicline product supplied. Ensure the patient understands the dosing schedule and the importance of adherence.
Follow-up advice to be given to patient or carer	Set your quit date on day 8-14 of treatment (when you start taking 1mg twice daily) Nausea is common initially but usually settles; taking the tablet with food may help Report any mood changes, depression, anxiety, or suicidal thoughts immediately to your healthcare provider or GP Contact your GP or healthcare provider if you experience chest pain, shortness of breath, or severe headaches Continue taking varenicline even if you have a slip-up (single cigarette); discuss with your pharmacist or GP Avoid driving or operating machinery if you experience dizziness or somnolence Combine varenicline with behavioural support (smoking cessation counselling, quitlines) for best results

	Attend follow-up appointments at 2 weeks, 4 weeks, and monthly to monitor progress and side effects If you become pregnant, inform your GP or healthcare provider immediately Do not stop taking varenicline suddenly; discuss any changes with your GP

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance: Smoking Cessation
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date		
30/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	**Change details**	**Date**
001	Development & issue of new PGD	1st November 2025

Version and change record

Version: v006

Issued: 30 September 2026

Supersedes: Version 005, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v005

Issued: 14 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Inclusion criteria: the quit date criterion now matches the dose row (day 8 to 14 of treatment).

Quantity row: the starter pack is described as the UK generic 2-week pack (11 x 0.5mg and 14 x 1mg); a 4-week starter pack does not exist.

Records row: the two run-together bullets were separated and punctuated (date of supply, dose, form, route and quantity supplied; adverse reactions and actions taken; supplied via PGD).

Records row: the under-18 record-retention bullet was removed because this PGD does not treat anyone under 18.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Product naming corrected: Champix was withdrawn from the UK market in 2021 and has not returned; the PGD names generic varenicline tablets, which are what pharmacies hold. 'Administration' corrected to 'supply' in the title: varenicline is supplied, not administered.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

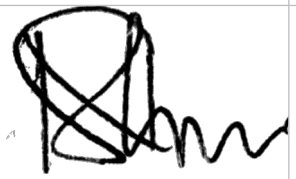
Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded.

Renal impairment: eGFR under 30 was both an exclusion and a caution carrying a dose the SmPC does not give. Now an exclusion only; eGFR 30 to 50 carries the SmPC advice (no adjustment; reduce to 1 mg once daily if not tolerated).

Signed on behalf of Get Real Health

Version v006, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.